

NAME : Mr.SIVA PRASAD	UHID NO/Visit ID : APF117.00003639/APF117.3639
Age/Gender : 22 Y 0 M 0 D /M	Collected : 25/Jun/2025 02:52PM
Ref Doctor : SELF	Received : 25/Jun/2025 03:58PM
Ref. Cust : SRINIVASA CLINICAL LAB	Reported : 25/Jun/2025 04:57PM
Client Code : APF117	Barcode : D3304328

DRL HEALTH CHECK - AP

Test Name	Result	Unit	Bio. Ref. Range	Method
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COMPLETE BLOOD COUNT (CBC) - 25 TESTS

Haemoglobin	15.0	gm%	13 - 17	Spectrophotometry
Total WBC Count	6710	Cells/cumm	4000 - 11000	Electrical Impedance
RBC Count	5.43	Millions/cumm	4.5 - 5.9	Electrical Impedance
Platelet Count	237	10 ³ /μL	150 - 450	Electrical Impedance
Packed Cell Volume (PCV)	45.2	%	40 - 50	Calculated
Mean Corpuscular Hb. (MCH)	27.7	pg	30 - 36	Calculated
Mean Corpuscular Volume (MCV)	83.4	fL	80 - 100	Calculated
MCHC	33.2	g/dL	32 - 36	Calculated
MPV	9.3	fL	7 - 11	Calculated
Platelet Crit	0.221	%	0.15 - 0.62	Calculated
RDW CV	13.2	%	11.5 - 14.5	Calculated
RDW-SD	41.10	fL	39.5 - 46.0	Calculated
PDW	16.4	fL	11 - 22	Calculated
P-LCR	39.0	%	18 - 50	Calculated

Differential Count by Flowcytometry/Microscopy

Neutrophils	55.1	%	45 - 75	Impedance/microscopy
Lymphocytes	34	%	20 - 40	Impedance/microscopy
Eosinophils	5.3	%	1 - 6	Impedance/microscopy
Monocytes	5.5	%	2 - 10	Impedance/microscopy
Basophils	0.1	%	0 - 2	Impedance/microscopy
Absolute Neutrophil Count	3697.21	cells/cumm	2000 - 7000	DHSS with Impedance
Absolute Basophils Count	6.71	Cells/cumm	0 - 100	DHSS with Impedance
Absolute Lymphocyte Count	2281.4	Cells/cumm	1000 - 3000	VCSn/Calculated
Absolute Eosinophil Count	355.63	Cells/cumm	50 - 500	DHSS with Impedance
Absolute Monocyte Count	369.05	Cells/cumm	200 - 1000	DHSS with Impedance
Mixed Cells	10.80	%		

Interpretation Notes:-

A complete blood count gives information regarding the cell types in person's blood and concentration of haemoglobin. Cells that circulate in the blood are generally divided into three types: RBC, WBC and platelets. Abnormally high or low counts may occur in physiological conditions and in diseased states and requires clinical correlation. Differential counts and RBC indices help in further understanding of the likely aetiology.

NOTE: This report has been generated by a fully automated analyser after counting thousands of cells and hence differential count may appear as decimalized numbers.


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GLYCOSYLATED HAEMOGLOBIN (GHB/HBA1C) , WHOLE BLOOD EDTA

Glycosylated Haemoglobin HbA1c	5.5	%	Normal: < 5.9 Pre-Diabetes: 5.9 - 6.4 Diabetes: > 6.5	HPLC
Approximate Mean Plasma Glucose	111.15			Calculated

Ref.for Biological Reference Intervals: American Diabetes Association.

Interpretation Notes:-

The A1C test measures your average blood glucose for the past 2 to 3 months. Diabetes is diagnosed at an A1C of greater than or equal to 6.5%.

Therapeutic goals for glycemic control (ADA):

-Adults:

- ◆ Goal of therapy: < 7.0% HbA1c
- ◆ Action suggested: > 8.0% HbA1c

-Pediatric patients:

- ◆ Toddlers and preschoolers: < 8.5% (but > 7.5%)
- ◆ School age (6-12 years): < 8%
- ◆ Adolescents and young adults (13-19 years): < 7.5%

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EGFR - GLOMERULAR FILTRATION RATE , SERUM

Creatinine	0.84	mg/dL	Males: 0.70 - 1.20	Modified Jaffe Kinetic
eGFR - Glomerular Filtration Rate	126.45	mL/min/1.73m ²	Refer Below	Calculated by using CKD EPI 2021 Formula

Biological Reference Ranges

Normal : >90 mL/min/1.73 m ²
Mildly Decreased : 60-90 mL/min/1.73 m ²
Mildly to Moderately Decreased: 45-59 mL/min/1.73 m ²
Moderately to Severely Decreased: 30-44 mL/min/1.73m ²
Severely Decreased: 15-29 mL/min/1.73m ²
Kidney Failure: <15 mL/min/1.7m ²

A normal eGFR for adults is greater than 90 mL/min/1.73m², according to the National Kidney Foundation. (Because the calculation works best for estimating reduced kidney function, actual numbers are only reported once values are less than 60 mL/min/1.73m²).

An eGFR below 60 mL/min/1.73m² suggests that some kidney damage has occurred. The test may be repeated to see if the abnormal result persists. Chronic kidney disease is diagnosed when a person has an eGFR less than 60 mL/min/1.73m² for more than three months.

A person may have some kidney damage even with an eGFR greater than 90 mL/min/1.73m². Other evidence, such as increased urine albumin, may indicate some degree of kidney damage. Thus, a person's eGFR should be interpreted in relation to the person's clinical history and present conditions.


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THYROID PROFILE (TFT) -I , SERUM

Triiodothyronine Total (TT3)	96.40	ng/dL	80 - 200	ECLIA
Thyroxine (TT4)	6.82	µg/dL	5.1-14.1	ECLIA
Thyroid Stimulating Hormone (TSH)	0.95	µIU/mL	0.27 - 4.2	ECLIA

Interpretation Notes:-

TSH	Total T4	Total T3	Disease
Normal	Normal	Normal	None
Low	High	High	Hyperthyroidism
Low	Normal	Normal	Subclinical Hyperthyroidism
Low	Normal	High	T3 Toxicosis
Low	High	Normal	Thyroiditis, T4 ingestion, hyperthyroidism in the elderly or with comorbid illness
Low	Low	Low	Euthyroid sick syndrome; central hypothyroidism
High	Normal	Normal	Subclinical hypothyroidism; recovery from euthyroid sick syndrome

- TSH levels are subject to circadian variation, reaching peak levels between 2 - 4 AM and at a minimum between 6 - 10 PM. The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations.
- Alteration in concentration of Thyroid hormone binding protein can profoundly affect Total T3 and/or Total T4 levels especially in pregnancy and in patients on steroid therapy.
- Unbound fraction (Free,T4/Free,T3) of thyroid hormone is biologically active form and correlate more closely with clinical status of the patient than total T4/T3 concentration.
- Values <0.03 µIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals.


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RENAL FUNCTION TEST (RFT) , SERUM

Urea	17.9	mg/dL	16.6 - 48.5	Urease
Creatinine	0.84	mg/dL	Males: 0.70 - 1.20	Modified Jaffe Kinetic
BUN	8.4	mg/dL	7 - 18	Calculated
BUN/Cr Ratio	9.95			Calculated
Uric Acid	6.16	mg/dL	Male: 3.4 - 7.0 Female: 2.4 - 5.7	Uricase
Sodium	140.1	mmol/L	135 - 150	ISE
Potassium	4.3	mmol/L	3.5 - 5.0	ISE
Chloride	98.9	mmol/L	94 - 110	ISE
Calcium	9.4	mg/dL	8.6 - 10.0	BAPTA

Interpretation Notes:-

Kidney/Renal function tests (KFT/RFT) are usually ordered when a patient has risk factors for kidney dysfunction such as hypertension, diabetes, cardiovascular disease, obesity, elevated cholesterol or a family history of kidney disease. It may also be ordered when someone has signs and symptoms of kidney disease, though in early stage often no noticeable symptoms are observed. Kidney panel is useful for general health screening; screening patients at risk of developing kidney disease; management of patients with known kidney disease.


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LIVER FUNCTION TEST (LFT) , SERUM

Total Bilirubin	0.50	mg/dL	< 1.2	Diazo
Direct Bilirubin	0.20	mg/dL	0 - 0.3	Diazo
Indirect Bilirubin	0.30	mg/dL	0.3 - 1.0	Calculated
SGOT / AST	24.2	U/L	< 40.0	IFCC
SGPT / ALT	50.0	U/L	< 42.0	IFCC
SGOT (AST) : SGPT (ALT) Ratio	0.48	Ratio	Refer Interpretation Notes	Calculated
Alkaline Phosphatase	67.3	U/L	40 - 129	IFCC
Total Protein	7.82	g/dL	6.6 - 8.7	Biuret
Albumin	5.06	g/dL	3.5 - 5.2	Bromocresol green
Globulin	2.76	g/dL	2.5 - 4.5	Calculated
A/G Ratio	1.83	Ratio	1.0 - 2.1	Calculated
Gamma Glutamyl Transferase (GGT)	23.10	U/L	Men: 8 - 61 Women : 5 - 36	Enzymatic colorimetric

Interpretation Notes:-

The most common liver tests include:

Liver enzymes test: Your liver enzymes include alkaline phosphatase (ALP), alanine transaminase (ALT), aspartate aminotransferase (AST) and gamma-glutamyl transferase (GGT). These are elevated when there's liver injury.

Total protein test: A total protein test measures levels of protein in your blood. Your liver makes protein, and low protein levels may indicate that your liver isn't functioning optimally.

Bilirubin test: Bilirubin is a waste product that liver deposits in bile. Possible diagnoses may include: Fatty liver disease, Toxic hepatitis, Autoimmune hepatitis, Viral hepatitis (A, B or C), Hemochromatosis, Wilson's disease, Alpha-1 antitrypsin deficiency, Primary biliary cholangitis (PBC), Cirrhosis. Liver cancer.

SGOT (AST) : SGPT (ALT) Ratio: A normal AST:ALT ratio should be <1. In patients with alcoholic liver disease, the AST:ALT ratio is >1 in 92% of patients, and >2 in 70%. AST:ALT scores >2 are, therefore, strongly suggestive of alcoholic liver disease and scores <1 more suggestive of NAFLD/NASH.


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LIPID PROFILE , SERUM

Total Cholesterol	175	mg/dL	Desirable : < 200 Borderline High :200 - 239 High : ≥ 240	Enzymatic colorimetric
HDL Cholesterol	35.0	mg/dL	Low: < 40, High: > 60	Enzymatic colorimetric
Total Triglycerides	156	mg/dL	Desirable Level : 150 Borderline : 150-199 High : 200-499 Very High : 500	Enzymatic colorimetric
VLDL Cholesterol	31.2	mg/dL	≤ 30	Calculated
LDL Cholesterol	108.8	mg/dL	Optimal: < 100 Above Optimal: 100 - 129 Borderline: 130 - 159 High: 160 - 189 Very High: ≥ 190	Calculation
Non - HDL Cholesterol	140	mg/dL	≤ 130	Calculated
Chol / HDL Ratio	5.0		Low Risk: 3.3 - 4.4 Average Risk: 4.5 - 7.1 Moderate Risk: 7.2 - 11.0 High Risk : > 11.0	Calculated
TGL / HDL Ratio	4.5		Optimal: < 2.5 Mild to moderate risk: 2.5 - 5.0 High Risk: > 5.0	Calculated
HDL / LDL Ratio	0.3			Calculated
LDL / HDL Ratio	3.1		Desirable: 0.5 - 3.0 Borderline Risk: 3.0 - 6.0 High Risk: > 6.0	Calculated

Interpretation Notes:-

Lipid profile measures the amount of cholesterol and fats called triglycerides in the blood. These measurements give the doctor a quick snapshot of what's going on in blood. Cholesterol and triglycerides in the blood can clog arteries, making you more likely to develop heart disease.


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BASIC IRON PROFILE

Iron	71.3	µg/dL	33 - 193	Colorimetric
Total Iron Binding Capacity	340.3	µg/dL	250 - 450	Chromogen Ferene
Transferrin Saturation%	20.95		12 - 50	Calculation
Unsaturated Iron Binding Capacity (UIBC)	269	µg/dL	Males: 125 - 345	Ferro Zine colorimetric

Interpretation Notes:-

Serum iron, total iron-binding capacity, and percent saturation are widely used for the diagnosis of iron deficiency. Symptoms of iron levels that are too low include:

- Pale skin
- Fatigue
- Weakness
- Dizziness
- Shortness of breath
- Rapid heart beat

Symptoms of iron levels that are too high can vary and tend to get worse over time. Symptoms may include:

- Joint pain
- Abdominal pain
- Lack of energy
- Weight loss


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DRL HEALTH CHECK - AP

Test Name	Result	Unit	Bio. Ref. Range	Method
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HIGH SENSITIVE CRP (HSCR) , SERUM

High Sensitive CRP (hsCRP)	2.96	mg/L	Relative risk: Low: < 1.0 Average: 1.0- 3.0 High risk: > 3.0	Immunoturbidometry
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Interpretation Notes:-

High Sensitivity C-reactive protein (hs-CRP) is often tested in a cardiovascular risk profile (good marker for inflammation), often along with Lipid profile. The CDC and the American Heart Association (AHA) recommend that measurement of hs-CRP greater than 10 mg/L be repeated twice, optimally two weeks apart, fasting or non-fasting in patients free of infection or acute illness.

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DRL HEALTH CHECK - AP

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VITAMIN - B12 , SERUM

Vitamin - B12	220	pg/mL	<1 year: 293 - 1210 2-3 year: 264 - 1215 4 - 6 year: 245 - 1075 7 - 9 year: 271 - 1170 10 - 12 year: 183 - 1090 13 - 18 year: 214 - 864 Adults: 191 - 663	ECLIA
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Interpretation Notes:-

Vitamin B12 and folate are critical to normal DNA synthesis, which in turn affects erythrocyte maturation. Vitamin B12 is also necessary for myelin sheath formation and maintenance. The body uses its B12 stores very economically, reabsorbing vitamin B12 from the ileum and returning it to the liver so that very little is excreted. Clinical and laboratory findings for B12 deficiency include neurological abnormalities, decreased serum B12 levels, and increased excretion of methylmalonic acid. The impaired DNA synthesis associated with vitamin B12 deficiency causes macrocytic anemias. These anemias are characterized by abnormal maturation of erythrocyte precursors in the bone marrow, which results in the presence of megaloblasts and in decreased erythrocyte survival. Pernicious anemia is a macrocytic anemia caused by vitamin B12 deficiency that is due to lack of intrinsic factor. Low vitamin B12 intake, gastrectomy, diseases of the small intestine, malabsorption, and trans-cobalamin deficiency can also cause vitamin B12 deficiency.

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25 (OH) VITAMIN-D , SERUM

25 (OH) Vitamin-D	24.80	ng/mL	Deficient: ≤ 20 Insufficiency: 21-29 Desirable: ≥ 30-100	ECLIA
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Interpretation Notes:-

Vitamin D deficiency (not enough vitamin D). These symptoms include

- Bone weakness & Softness
- Bone malformation (in children)
- Fractures

Higher risk for a vitamin D deficiency.

- Osteoporosis or other bone disorder
- Previous gastric bypass surgery
- Age; vitamin D deficiency is more common in older adults.
- Obesity
- Lack of exposure to sunlight
- Having a darker complexion
- Difficulty absorbing fat in your diet

In addition, breastfed babies may be at a higher risk if they aren't taking vitamin D supplements.

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GLUCOSE - FASTING (FBS) , NAF PLASMA

Fasting Glucose	89.6	mg/dL	70 - 110	Hexokinase
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Ref.for Biological Reference Intervals: American Diabetic Association.

Interpretation Notes:-

A blood glucose test measures the glucose levels in your blood. Glucose is a type of sugar. It is your body's main source of energy. Symptoms of high blood glucose levels include increased thirst, more frequent urination, Blurred vision, Fatigue, Wounds that are slow to heal. Symptoms of low blood glucose levels include Anxiety, Sweating, Trembling, Hunger and Confusion. Blood glucose test is required to check certain risk factors for diabetes. These include Being overweight, Lack of exercise, Family member with diabetes, High blood pressure, Heart disease.


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HOMOCYSTEINE , SERUM

Homocysteine	40.20	umol/L	6-22	CLIA
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Interpretation Notes:-
Several clinical and epidemiological studies have indicated that even a moderately elevated plasma HCY level is a predictor for cardiovascular disease. Homocysteine levels also have been found to correlate to the extent of atherosclerotic plaque in the aorta. Homocysteine can impair the ability to repair vascular endothelial cell injury, and thus promote the development of atherosclerosis. A 5-year prospective study on the combined effect of hyperhomocysteinemia and type-2 diabetes on mortality in 2484 men and women showed that hyperhomocysteinemia is related to mortality independent of other cardiovascular risk factors and appeared to be a stronger (1.9 fold) risk factor for mortality in type-2 diabetic patients than in nondiabetic subjects.


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FERRITIN , SERUM

Ferritin	84.30	ng/mL	30 - 400	ECLIA
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Interpretation Notes:-

Symptoms of ferritin low levels include:

- Pale skin
- Fatigue
- Weakness
- Dizziness
- Shortness of breath
- Rapid heart beat
- Restless legs syndrome, a condition that may be related to low iron levels.

Symptoms of ferritin high levels include:

- Joint pain
- Abdominal pain
- Lack of energy
- Weight loss

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INSULIN , SERUM

Insulin	27.59	uU/mL	2.6 - 24.9	ECLIA
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Interpretation Notes:-

An insulin in blood test is most often used to find out:

- The cause of hypoglycemia (low blood sugar)
- Diagnose or monitor insulin resistance
- Monitor the condition of people with type 2 diabetes
- Find out if there is a type of tumor on the pancreas, known as an insulinoma. If the tumor has been removed, the test may be used to see if it has been done successfully.

An insulin in blood test is sometimes used along with other tests to help diagnose and monitor type 1 diabetes. These other tests may include glucose and hemoglobin A1C testing.

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CUE - COMPLETE URINE ANALYSIS (AUTOMATED REAGENT STRIP METHOD AND MICROSCOPY) , URINE

Physical Examination				
Volume	NA	mL		
Colour	Yellow		Pearly white	
Appearance	Clear		Clear	
pH	6.0		5.0 - 8.5	Dipstix-Double indicator
Specific Gravity	1.015		1.005 - 1.030	Dipstix-Ion exchange
Chemical Examination				
Albumin Urine/ Protein Urine	Negative		Negative	Dipstix-Bromophenol blue
Glucose	Negative		Negative	Dipstix-Oxidase peroxidase
Ketone Bodies	Negative		Negative	Dipstix-Sodium nitroprusside
Bilirubin	Negative		Negative	Dipstix-Coupling/Fouchet
Urobilinogen	Negative		Negative	Dipstix-Ehrlich's reaction
Blood	Negative		Negative	Dipstix-Peroxidase
Nitrite	Negative		Negative	Dipstix-Greiss
Leukocyte esterase	Negative		Negative	Dipstix-Esterase
Microscopic Examination				
Pus Cells (Leucocytes)	2-3	/hpf	0 - 5	Microscopy
Epithelial Cells	1-2	/hpf	0 - 5	Microscopy
RBCs	Nil	/hpf	Nil	Microscopy
Casts	Nil		Nil	Microscopy
Crystals	Nil		Nil	Microscopy
Bacteria	None seen		None seen	Microscopy
Budding Yeast Cells	None seen		None seen	Microscopy
Others	Nil		Nil	Microscopy

Printed On :25-Jun-2025 06:31 PM, Sample Processed at : CHENNAI,

*** End Of Report ***

Disclaimer:


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Consultant Pathologist



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NAME : Mr.SIVA PRASAD	UHID NO/Visit ID : APF117.00003639/APF117.3639
Age/Gender : 22 Y 0 M 0 D /M	Collected : 25/Jun/2025 02:52PM
Ref Doctor : SELF	Received : 25/Jun/2025 04:03PM
Ref. Cust : SRINIVASA CLINICAL LAB	Reported : 25/Jun/2025 06:20PM
Client Code : APF117	Barcode : D3304326

Test Name	Result	Unit	Bio. Ref. Range	Method
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1. It is presumed that the specimen belongs to the patient named or identified in the test request form. 2. Results pertain to the specimen submitted and all results are dependant on the quality of sample received in the laboratory. 3. The test results are for information and interpretation of the referring doctor or other medical professional who understands reporting units, reference ranges and limitations of terminologies.4. Laboratory investigations help in arriving at a diagnosis and should be clinically correlated.5. This report is not valid for medico-legal purposes. 6. Partial reproduction of this report is not permitted/not valid.


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